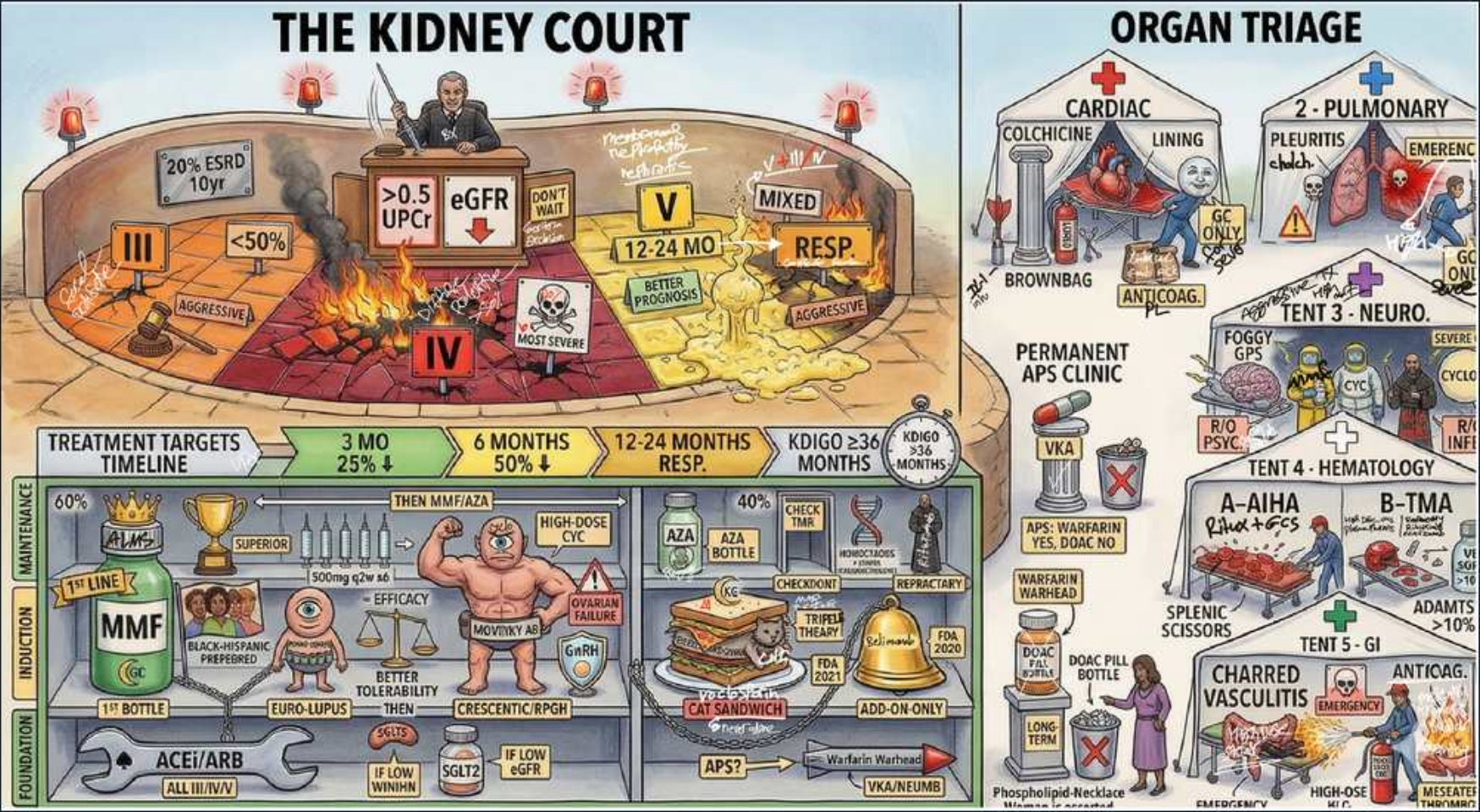




1. The Kidney Court

WHAT YOU SEE

A THE KIDNEY COURT banner over a courtroom with a judge — the biopsy is the judge that decides the ISN/RPS class and therefore the treatment.

WHAT IT ENCODES

Lupus nephritis arises from immune-complex (anti-dsDNA/DNA) deposition in the glomerulus and is classified by the ISN/RPS system into six histologic classes (I–VI) on renal BIOPSY. The 'court' metaphor: the biopsy is the judge that decides treatment. Class drives therapy — proliferative classes (III/IV) need aggressive induction immunosuppression, whereas pure membranous (V) and minimal/mesangial (I/II) are managed differently. LN occurs in ~40–60% of SLE patients and is a major driver of morbidity.

BOARD TRAP

WRONG: assigning a lupus nephritis class or starting class-specific therapy based on serology or proteinuria alone. Discriminator: only the renal BIOPSY defines the ISN/RPS class — anti-dsDNA, complement, and proteinuria suggest activity but cannot distinguish proliferative (III/IV) from membranous (V) disease, which require different treatment.

2. Class III (focal)

WHAT YOU SEE

A burning panel marked III with <50% — flames = active glomerular inflammation; 'focal' means under half the glomeruli, but still aggressive.

WHAT IT ENCODES

Class III is FOCAL proliferative lupus nephritis: endocapillary proliferation/inflammation involving <50% of glomeruli. Despite being 'focal,' it is an aggressive immune-complex glomerulonephritis presenting with hematuria, RBC casts, proteinuria, and possible renal impairment. Like class IV, it requires INDUCTION immunosuppression (steroids + MMF or cyclophosphamide). The flames signal active inflammation.

BOARD TRAP

WRONG: undertreating class III as 'mild' because only a minority of glomeruli are involved. Discriminator: focal (III) and diffuse (IV) proliferative LN are both treated with aggressive induction immunosuppression — the 'focal' label refers to glomerular extent, not to disease severity, and does not justify a less intensive regimen.

3. Class IV (diffuse, most severe)

WHAT YOU SEE

The panel marked IV with the LARGEST flames and a MOST SEVERE tag — biggest fire = diffuse proliferative disease, the worst and commonest class.

WHAT IT ENCODES

Class IV is DIFFUSE proliferative lupus nephritis (≥50% of glomeruli) — the MOST COMMON and MOST SEVERE class, with the highest risk of progression to ESRD. It presents with active nephritic sediment (RBC casts, dysmorphic RBCs), heavy proteinuria, hypertension, low complement, and high anti-dsDNA. Treatment is the most aggressive: high-dose glucocorticoids plus MMF or cyclophosphamide for induction. The biggest flames = worst class.

BOARD TRAP

WRONG: undertreating class IV or assuming it self-limits. Discriminator: diffuse proliferative (IV) lupus nephritis is the most aggressive class and the leading renal cause of ESRD in SLE — it mandates full induction immunosuppression (pulse steroids + MMF or low-dose cyclophosphamide), and undertreatment leads to irreversible renal failure.

4. Proliferative III/IV → biopsy + IS

WHAT YOU SEE

AGGRESSIVE labels stamped across the burning III/IV panels — proliferative classes demand full induction immunosuppression, not HCQ/ACEi alone.

WHAT IT ENCODES

Proliferative lupus nephritis (classes III and IV) is the treatment-defining group: confirm by biopsy, then give INDUCTION immunosuppression — pulse IV methylprednisolone followed by oral steroids, PLUS MMF (~2–3 g/day) or cyclophosphamide, increasingly with add-on belimumab or a calcineurin inhibitor (voclosporin) for better response. Adjuncts: ACEi/ARB, hydroxychloroquine for all, and strict BP control. Induction runs ~6 months, then maintenance.

BOARD TRAP

WRONG: treating proliferative LN with hydroxychloroquine and ACEi alone, or with steroids alone. Discriminator: HCQ and ACEi/ARB are essential ADJUNCTS but are NOT sufficient for class III/IV — proliferative LN requires combination INDUCTION immunosuppression (steroids + MMF or cyclophosphamide ± belimumab/CNI) to prevent ESRD.

5. Class V (membranous)

WHAT YOU SEE

A panel marked V with a worse-prognosis note — membranous disease causing nephrotic-range proteinuria; often coexists with III/IV as 'mixed'.

WHAT IT ENCODES

Class V is MEMBRANOUS lupus nephritis — subepithelial immune-complex deposits causing NEPHROTIC-range proteinuria, edema, hypoalbuminemia, hyperlipidemia, and a hypercoagulable (thrombotic) state. Pure class V is treated with steroids plus an immunosuppressant (MMF, a calcineurin inhibitor/voclosporin, or others) and full antiproteinuric/RAAS blockade. Class V frequently COEXISTS with III or IV ('mixed' membranous + proliferative), which is then treated as proliferative disease.

BOARD TRAP

WRONG: treating membranous LN (V) exactly like proliferative LN, or missing a coexisting proliferative component. Discriminator: pure class V is nephrotic (not nephritic) and does not always need cyclophosphamide, but MIXED class V+III/IV must be treated with full proliferative-grade induction — so the biopsy must be read for a mixed pattern, and watch for nephrotic-related venous thrombosis.

6. Proteinuria >0.5 / eGFR

WHAT YOU SEE

A courtroom sign reading >0.5 UPCr and eGFR — these thresholds (proteinuria >0.5 g/g, active sediment, falling eGFR) are the triggers that summon a biopsy.

WHAT IT ENCODES

Renal biopsy is indicated for proteinuria >0.5 g/day (UPCr >0.5 g/g), an active urinary sediment (RBC casts, dysmorphic RBCs, cellular casts), or otherwise unexplained decline in eGFR/rising creatinine in an SLE patient. These thresholds catch clinically significant nephritis early, before irreversible damage. Ongoing monitoring tracks UPCr, creatinine/eGFR, anti-dsDNA, and complement.

BOARD TRAP

WRONG: deferring biopsy in a lupus patient with new proteinuria >0.5 g/day or active sediment, or treating empirically. Discriminator: these specific triggers (UPCr >0.5, active sediment, or unexplained eGFR drop) should prompt biopsy to define class BEFORE therapy — empiric treatment risks mismatching the regimen to the histology.

7. MMF first-line induction

WHAT YOU SEE

An MMF trophy with a 'Black/Hispanic preferred' tag — the gold-cup first-line induction agent, especially favored in those groups.

WHAT IT ENCODES

Mycophenolate mofetil (MMF) is a preferred FIRST-LINE induction agent for proliferative lupus nephritis — equivalent efficacy to cyclophosphamide with better tolerability and no infertility risk. It is particularly favored in Black and Hispanic patients, who respond better to MMF than to cyclophosphamide. Combined with steroids (and increasingly belimumab or voclosporin) it forms modern induction. The trophy marks it as the go-to.

BOARD TRAP

WRONG: continuing MMF in a patient who becomes pregnant or is planning conception. Discriminator: MMF is TERATOGENIC and contraindicated in pregnancy — switch to AZATHIOPRINE (pregnancy-compatible) before conception. MMF is first-line for non-pregnant LN induction, but pregnancy flips the choice to AZA.

8. Cyclophosphamide option

WHAT YOU SEE

A muscular strongman bottle for CYC induction — the brute-force alternative for severe/crescentic disease (needs MESNA + ovarian protection).

WHAT IT ENCODES

Cyclophosphamide is the alternative INDUCTION agent for proliferative lupus nephritis, especially severe/crescentic or rapidly progressive disease. The low-dose 'Euro-Lupus' regimen (500 mg IV every 2 weeks × 6 doses) gives efficacy comparable to high-dose NIH regimens with less toxicity. Protect against its key harms: hemorrhagic cystitis/bladder cancer (MESNA + hydration), gonadal failure (GnRH-agonist ovarian protection), myelosuppression, and infection.

BOARD TRAP

WRONG: choosing high-dose cyclophosphamide by default or omitting bladder/fertility protection. Discriminator: low-dose Euro-Lupus CYC matches high-dose efficacy with fewer adverse effects, and any CYC requires MESNA + hydration (hemorrhagic cystitis/bladder cancer) and ovarian protection (GnRH agonist) — MMF is generally preferred unless disease is severe/crescentic.

9. ACEi/ARB for all LN

WHAT YOU SEE

An ACEi/ARB foundation block stamped ALL — the antiproteinuric base layer beneath all therapy, but never a substitute for immunosuppression.

WHAT IT ENCODES

An ACE inhibitor or ARB is foundational ('ALL') for proteinuric lupus nephritis: it reduces intraglomerular pressure and proteinuria and controls blood pressure, slowing CKD progression. It is an adjunct layered beneath immunosuppression along with hydroxychloroquine, BP control, and cardiovascular risk reduction. Monitor potassium and creatinine after initiation.

BOARD TRAP

WRONG: relying on ACEi/ARB as the treatment for active nephritis, or using both together. Discriminator: RAAS blockade is an essential ADJUNCT (antiproteinuric, renoprotective) but does NOT treat the underlying immune-complex inflammation — proliferative LN still needs immunosuppression. Avoid ACEi/ARB combination therapy, and avoid in pregnancy (teratogenic).

10. Azathioprine maintenance

WHAT YOU SEE

An AZA bottle in the maintenance phase of the timeline — the lower-intensity drug that prevents relapse and is the pregnancy-safe maintenance choice (check TPMT).

WHAT IT ENCODES

After ~6 months of induction and response, therapy transitions to lower-intensity MAINTENANCE for ≥3 years to prevent relapse — MMF or azathioprine, with steroids tapered to the lowest effective dose. MMF is the more effective maintenance agent overall, but AZATHIOPRINE is the preferred maintenance drug in pregnancy. Check TPMT/NUDT15 activity before azathioprine to avoid severe myelosuppression.

BOARD TRAP

WRONG: stopping immunosuppression once induction achieves remission, or starting azathioprine without checking TPMT. Discriminator: maintenance immunosuppression must continue for years to prevent relapse; AZA is the pregnancy-safe maintenance choice, and TPMT/NUDT15 deficiency causes life-threatening myelosuppression — so test before dosing AZA.

11. Cardiac — pericarditis

WHAT YOU SEE

A CARDIAC tent with COLCHICINE and a heart 'lining' note — pericarditis (the inflamed lining) is the commonest cardiac sign; also watch Libman-Sacks endocarditis.

WHAT IT ENCODES

Pericarditis (serositis of the heart's 'lining') is the MOST COMMON cardiac manifestation of SLE, presenting with pleuritic chest pain relieved by leaning forward, a friction rub, and diffuse ST elevation. Treat with NSAIDs, colchicine, and steroids for refractory cases. Lupus also causes Libman-Sacks (non-bacterial verrucous) endocarditis, myocarditis, and accelerated coronary atherosclerosis (a leading late cause of death).

BOARD TRAP

WRONG: stopping at pericarditis and forgetting the other cardiac risks. Discriminator: pericarditis is most common, but Libman-Sacks ENDOCARDITIS (sterile valvular vegetations, linked to antiphospholipid antibodies, embolic risk) and PREMATURE coronary artery disease are high-yield — lupus patients die of accelerated atherosclerosis, so aggressive CV risk modification matters.

12. Pulmonary — pleuritis

WHAT YOU SEE

A PULMONARY tent marked PLEURITIS with an EMERGENT alveolar-hemorrhage warning — pleuritis is common, but rule out infection before blaming lupus pneumonitis.

WHAT IT ENCODES

Pleuritis (serositis) is the most common pulmonary manifestation — pleuritic chest pain with possible exudative effusions. Far rarer but EMERGENT are acute lupus pneumonitis and diffuse alveolar hemorrhage (hemoptysis, dropping hemoglobin, diffuse infiltrates), which are life-threatening and require pulse steroids ± cyclophosphamide/rituximab and plasma exchange. 'Shrinking lung syndrome' (diaphragmatic dysfunction) is a rare restrictive complication.

BOARD TRAP

WRONG: attributing every new infiltrate in a lupus patient to lupus pneumonitis. Discriminator: INFECTION is far more common than lupus pneumonitis in an immunosuppressed lupus patient and must be excluded first; reserve high-dose immunosuppression for true alveolar hemorrhage/pneumonitis after ruling out infection.

13. Neuro / APS — anticoagulate

WHAT YOU SEE

A PERMANENT APS CLINIC tent with a VKA/WARFARIN sign — APS thrombosis gets lifelong warfarin (anticoagulate, NOT immunosuppress).

WHAT IT ENCODES

Neuropsychiatric SLE spans seizures, psychosis, cognitive dysfunction, and stroke. A critical board distinction: thrombotic events from secondary ANTIPHOSPHOLIPID SYNDROME are treated with ANTICOAGULATION, not immunosuppression. After a thrombotic event in APS, give lifelong warfarin (target INR 2–3; higher or dual therapy for arterial/recurrent events). DOACs are inferior in APS, especially triple-positive/arterial disease.

BOARD TRAP

WRONG: immunosuppressing an APS-related stroke or using a DOAC for thrombotic APS. Discriminator: APS thrombosis → lifelong WARFARIN (vitamin-K antagonist), as DOACs are less effective in APS (notably triple-positive/arterial); only true inflammatory NPSLE (cerebritis, myelitis) warrants immunosuppression — distinguish thrombotic from inflammatory neuro disease.

14. Hematology — AIHA & TMA

WHAT YOU SEE

Two hematology tents A-AIHA (Coombs+, splenic scissors) and B-TMA (schistocytes) — distinguishing Coombs-positive hemolysis from microangiopathy that needs plasma exchange.

WHAT IT ENCODES

SLE causes several cytopenias: warm autoimmune hemolytic anemia (DAT/Coombs POSITIVE, spherocytes, high LDH/indirect bilirubin, low haptoglobin), immune thrombocytopenia (ITP, sometimes the presenting feature), leukopenia/lymphopenia, and thrombotic microangiopathy (TMA — schistocytes, thrombocytopenia, often overlapping TTP or catastrophic APS). Severe AIHA/ITP is treated with steroids ± rituximab; TMA may need plasma exchange.

BOARD TRAP

WRONG: conflating AIHA with TMA or missing TTP/CAPS. Discriminator: AIHA is Coombs-POSITIVE hemolysis WITHOUT schistocytes, whereas TMA/TTP shows SCHISTOCYTES with a NEGATIVE Coombs and needs urgent plasma exchange — distinguishing immune hemolysis from microangiopathic hemolysis changes management dramatically.